



DEPARTMENT OF MEDICAL ONCOLOGY

DISCHARGE SUMMARY SS UNIT

Patient Name	: Mrs. DHANMANTI	UHID	: 300117827
Bed No	: 165-2-DC	IP. No.	: AC26/7736
Company	: SHA (U.P) - Credit	Age/Gender	: 52 Yrs 4 Mths 14 Days/Female
Contact No	: 8802540755	Admission Date	: 15/05/2026 01:37 PM
Unit Name	: Dr. SS UNIT	Date of Discharge:	15/05/2026
Doctor Name	: Dr. Sushant Mittal, Dr. Samit Purohit		
Address	: 7121 SARAI NAWAB ALIGARH, ALIGARH, Uttar Pradesh, INDIA		

DISCHARGE SUMMARY

1. DIAGNOSIS WITH STAGE:- CARCINOMA LEFT BREAST (POST NACT)
ER- POSITIVE PR - POSITIVE HER-2-NEU NEGATIVE
POST NACT
POST SURGERY (JAN 2022)

ADMITTED FOR : Chemotherapy

2. KNOWN ALLERGIES:-No known drug allergies

3. CO-MORBIDITIES: Nil

4. PAST HISTORY:- Nothing significant

5. HISTORY OF PRESENT ILLNESS: Mrs. Dhanmanti, 51 years old normotensive and nondiabetic female is a known case of Ca breast- LABC. She was investigated outside. Turcut Biopsy, Left breast lump done on 17/03/2021. FNAC- Left axillary lymph node. CECT Chest done on 04/04/2021 s/o a large heterogeneously enhancing soft tissue density lesion in the outer upper aspect of the breast parenchyma. It measures 45 x 38mm. It is lying close to the pectoralis major muscle. Mild streakiness is seen in that fat surrounding the lesion in the breast parenchyma. It represents known carcinoma. A soft tissue density lesion measuring 15 x 11 mm in the left anterior chest wall subcutaneous fat at the level of anterior end of the left 4th rib- It may represent satellite lesion. Multiple heterogeneously enhancing and enlarged lymphnodes in the left axilla. The lymphnodes measure upto 25 x 18 mm- Metastatic lymphnodes. A small soft tissue density lesion measuring 9 x 7 mm in the infra-axillary ft -?Lymphnode. Few fibro calcific lesion with costal pleural thickening seen in the apical segment of the right upper lobe. The lesion measures 3-8mm. These nodules may represent chronic infiltrates. PET-CT (05/04/2021)- No definite scan evidence of any osteoblastic skeletal metastasis in the present study. USG Guided Biopsy from Left Breast Mass done on 13/04/2021- Invasive breast carcinoma. ER- Positive, PR- Positive and Her2Neu- Negative.



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PET-CT Whole Body Scan (24/06/2021)- Soft tissue density mass with irregular margins and showing increased FDG uptake is seen in upper quadrants of left breast extending to central quadrant (3.9 x 3.1cmn, SUVmax6.4). Overlying skin appears mildly thickened with no significant FDG uptake. Nodular lesion with increased FDG uptake is seen along left anterior chest wall at the level of 5th rib, closely abutting the underlying chest wall muscles (1.3 x 1.1cm, SUVmax- 3.6). Multiple discrete and confluent left axillary lymph nodes with increased FDG uptake are seen, largest measuring- 2.6 x 1.7cm, SUVmax- 4.6. Few left infraclavicular lymph nodes with mildly increased FDG uptake are seen. Small left internal mammary lymph node with mildly increased FDG uptake is seen at the level of 3rd costal cartilage.

She received 4 cycles of (Inj. Adriamycin 70mg + Inj. Endoxan 700mg) MDCT + IDCT chemotherapy from 24/06/2021 to 28/08/2021

She received 10cycles of (Inj. Paclitaxel 230mg) chemotherapy from 20/09/2021 to 18/12/2021

She underwent Left modified radical mastectomy + axillary dissection + rorationplasty + fasciocutaneous flap done under GA on 20/01/2022.

Right cervical lymph node, USG guided biopsy(04/12/2024) s/o Metastatic carcinoma

She received 5 cycles of chemotherapy (Inj. Zolasta 4mg)(Targeted + Adjuvant therapy) till 25/08/2025

PET CT Scan (05/10/2025) reveals post left MRM status with absence of FDG avid disease in the operative bed. FDG avid right cervical, supra and infraclavicular, mediastinal, retroperitoneal and pelvic lymph nodes, peritoneal and intermuscular nodules/nodes with right pulmonary nodules and multiple skeletal lesions, likely metastatic disease. No other FDG avid metastatic disease noted elsewhere in rest of the body surveyed. Significant increase in extent of right hydroureteronephrosis noted. As compared to the previous PET CT



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Scan done on June 04, 2025, mild increase in size and metabolic activity of most of the metastatic lymph nodes noted. Re appearance of FDG avid intramuscular, peritoneal nodules noted in the pelvis. Few new FDG avid skeletal lesions are noted. The imaging overall suggest disease progression.

Right supraclavicular lymphnodes, Trucut biopsy (08/10/2025) Features of metastatic carcinoma, right supraclavicular lymphnode biopsy.

IHC (08/10/2025) Features of metastatic carcinoma, consistent with breast origin primary (known case of Ca breast).

(15/10/2025) ER: Positive, PR: Positive, HER-2: Negative.

She received C1-D1 of (Inj. Paclitaxel 130mg) (Targeted + Adjuvant chemotherapy) on 21/10/2025

She received C2-D1 of (Inj. Paclitaxel 130mg) (Targeted + Adjuvant chemotherapy) on 31/10/2025

She received C3-D1 of (Inj. Paclitaxel 130mg) (Targeted + Adjuvant chemotherapy) on 26/11/2025

She received C4-D1 of (Inj. Tortaxel 130mg) (Targeted + Adjuvant chemotherapy) on 05/12/2025

She received C5-D1 of (Inj. Tortaxel 130mg) (Targeted + Adjuvant chemotherapy) on 17/12/2025

She received C6-D1 of (Inj. Paclitaxel 130mg) (Targeted + Adjuvant chemotherapy) on 29/12/2025

She received C7-D1 of (Inj. Paclitaxel 130mg) (Targeted + Adjuvant chemotherapy) on 08/01/2026

She received C8-D1 of (Inj. Tortaxel 130mg) (Targeted + Adjuvant chemotherapy) on 19/01/2026

She received C9-D1 of (Inj. Tortaxel 130mg) (Targeted + Adjuvant chemotherapy) on 31/01/2026

She received C10-D1 of (Inj. Tortaxel 130mg) (Targeted + Adjuvant chemotherapy) on 11/02/2026

She received C1-D1 of (Inj. Zolexa 4mg) (Targeted + Adjuvant chemotherapy) on 12/02/2026. Inj. Peg F 6mg S/C on 12/02/2026.



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She received C11-D1 of (Inj. Tortaxel 130mg) (Targeted + Adjuvant chemotherapy) on 25/02/2026

She received C12-D1 of (Inj. Tortaxel 130mg) (Targeted + Adjuvant chemotherapy) on 16/03/2026

Inj. Peg F 6mcg s/c which was given on 17/03/2026.

She received C1 D1 of (Inj. Zolexa4mg) chemotherapy on 30/04/2026.

Presently she was admitted for chemotherapy. She received C13-D1 of (Inj. Paclitaxel 130mg) (Targeted + Adjuvant chemotherapy) on 15/05/2026 with prehydration, posthydration and antiemetics. Patient tolerated the treatment well and is now being discharged in stable condition.

6. FAMILY HISTORY:- Nothing significant

7. GENERAL EXAMINATION:- Patient Conscious, Oriented, Afebrile, BP-120/70mmHg, PR-70/min, RR-20/min., Chest-B/L AE+, CVS-S1S2+, P/A- Soft, BS+.

8. INVESTIGATIONS: Attached

9. CONDITION AT DISCHARGE: Satisfactory

10. ADVICE ON DISCHARGE:

TAB. PANCITA-DSR TWICE DAILY BEFORE MEALS

TAB. EMESET 8MG THRICE DAILY FOR 5 DAYS (BEFORE MEALS)

TAB. DOLOSWIFT TWICE DAILY FOR 5 DAYS

CYTOACT HP PROTEIN POWDER 2TSF TWICE DAILY

TAB. BONMAC ONCE DAILY FOR 7 DAYS

TAB. XYMEG 160MG TWICE DAILY FOR 7 DAYS



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SYP. LUXATIN 160MG AT BED TIME FOR CONSTIPATION
PLENTY OF FLUIDS /ADEQUATE HYDRATION

OTHER MEDICINES

CARDIAC/DIABETIC/THYROID MEDICINES TO CONTINUE AS ADVISED BY CONCERNED PHYSICIAN

IN CASE OF VOMITING

INJ. EMESET 8 mg IV SOS

IN CASE OF FEVER $>100^{\circ}\text{F}$, INFORMED IMMEDIATELY OR REPORT TO HOSPITAL

DIET ADVISED:- Neutropenic normal diet

Precautions:- As prescribed

- 11. FOLLOW UP:-** 1. REVIEW WITH DR. SAMIT PUROHIT / DR. SUSHANT MITTAL ON 22/05/2026 WITH CBC REPORT IN MEDICAL ONCOLOGY OPD.
2. FOR OPD APPOINTMENT CALL ON 011-49222222.

SPECIAL NEEDS:

MAJOR SUB ELEMENTS (To be filled / *Tick whichever is applicable)

Preventive Aspect (if ANY): Life Style, Balanced Diet, Regular Exercise, Stress Management, No Smoking,

Good Personal Hygiene, No Alcohol, Other(s) (if any).....Discharge

Planning: Oxygen Therapy / BIPAP Support / Post-op-Dressing / Physiotherapy / Intercostal Drainage (ICD) /

Ryle's Tube Feed / PEG Feed / FJFeed / Others



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IN CASE OF THE FOLLOWING MEDICAL PROBLEMS CONTACT YOUR DOCTOR

Persistent fever
Recurrent Vomiting
Bleeding from any site
Severe Abdominal pain etc

IN CASE OF EMERGENCY CALL CMO ON DUTY ON 011-49222220

Admission Waiting- Call Action Cancer Hospital Reception, Call- 011-49222222
For Appointment PET-CT Call- 011-49222222

In case of fever > 100° F - Do CBC

Take Crocin 500mg
Report to OPD or Emergency
PH No.- 011-49222222

In case of Loose Motions- Do CBC/ KFT

Tab. Ciplox-TZ Twice Daily (3 Days)
Cap. Vizylac Twice Daily
ORS/ Electral

DR. SAMIT PUROHIT
DIRECTOR - MEDICAL ONCOLOGY
MBBS, MD, DM


DR. SUSHANT MITTAL
DIRECTOR - MEDICAL ONCOLOGY
MBBS, MD, DM

H-2/FC-34, A-4, Paschim Vihar, New Delhi-110063

Tel.: 011-49-222-222, E-mail: ach@actioncancerhospital.com, Website: www.actioncancerhospital.com

pared By: Mr. Varun Arora

Finalize By: Mr. Varun Arora

A UNIT OF MANAV SEVARTH TRUST



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SAMITMEDONC@GMAIL.COM

SUSHANTMITTAL80@GMAIL.COM

For appointment or any query related to insurance or panel, contact Mr. Kiran, OPD coordinator (S.S. unit), Mobile: 8588868723, Landline 011-49222222(Extn:5824).

The above treatment had been explained to me along with medications

Name of Patient/Attendant:.....

Signature of Patient/Attendant:.....

Relation with Patient:-